One year. Different patterns.

Evelyn Brooks, 67 - entirely fictional - Nov 2025 to Oct 2026

495
generated activities

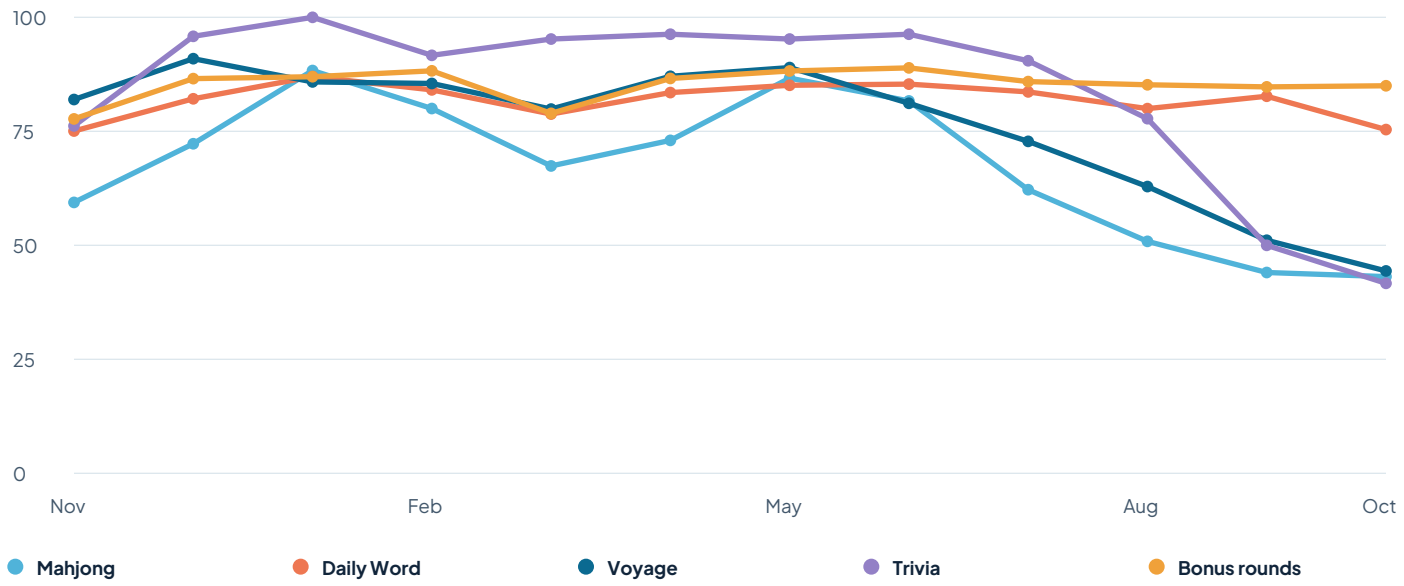
471
eligible check-ins

5,072
fabricated trials

The fictional year shows early task learning, a temporary dip in March, and later divergence. Matching, navigation and delayed recognition become less efficient, while the guided trail stays comparatively stable. These are deliberately scripted patterns, not evidence of Alzheimer's disease.

Five activities, five different curves

Monthly mean demo points / 100 - arbitrary activity-specific transforms



What the year makes visible

Early practice gains. A temporary March dip. Later separation between memory/navigation/recall and the steadier trail. This is an authored visual story, not disease natural history.

AI-authored interpretation, cached for the pitch. Source: evelyn-year-v2, seed 671104; scorer demo-v2. Raw records and sample counts remain inspectable.

From curves to measurements

Reference: pooled eligible sessions in November–January. Latest: October. The reference includes learning and is not a validated baseline.

Matching takes more attempts

5.8 to 9.6 attempts

n = 24 reference / 9 October

Memory Mahjong averages 5.8 pair attempts in the first three months and 9.6 in October. The placeholder score changes from 73.9 to 43.1. This illustrates a repeated performance difference; it does not identify a cause.

Animal naming changes more modestly

20.2 to 18.3 animals

n = 24 reference / 9 October

Unique recognized animals average 20.2 per 60 seconds in the first three months and 18.3 in October. A finite dictionary and input method affect the counts. This is a category-fluency game proxy.

Navigation separates from the other curves

83.7 to 41.5 %

n = 23 reference / 9 October

Route efficiency changes from 83.7% to 41.5%. October's mean pointing error is 95.0 degrees on an eight-choice compass. Device interaction, route familiarity and quantization matter; this is not validated path integration.

Delayed recognition also changes

90.9 to 41.7 %

n = 22 reference / 8 October

Quiz accuracy averages 90.9% in the first three months and 41.7% in October. October's mean delay is 97.1 seconds. Only three questions are asked per round, and the delay varies; both limit interpretation.

The guided trail stays comparatively stable

24.7 to 24.0 s

n = 22 reference / 9 October

Mean completion time changes from 24.7 to 24.0 seconds. That descriptive stability is useful context alongside the other curves. It is not proof that an attention or executive-function domain is intact.

Every measurement above is generated. Lower or higher values describe the game only; they do not establish a clinical change or its cause.

Keep the meaning honest.

What this demonstration can show

Repeated game observations can display different temporal patterns. Linking a narrative to raw features, denominators and protocol details makes the pattern understandable and reviewable.

What it cannot establish

1. Every person, response and trend is fabricated. The generator was designed to produce the visual story.

2. The 0–100 scores use arbitrary demo-v2 equations. No clinical norms, risk thresholds, percentiles or validated change criteria exist.

3. The November–January reference includes learning. Comparisons are descriptive means, without significance testing or diagnostic interpretation.

4. Practice, paused and assisted records are excluded from curves. Missing records are absent, never scored as zero.

5. Speech transcription, eight-way compass choices, a short guided trail and multiple-choice recall are simplified proxies.

A useful next conversation

1. Repeat the same task protocol and input method to build a comparable record.

2. Review raw measurements and collection quality alongside the curves.

3. In a future real-data setting, a person could choose to share a factual summary with a qualified clinician. This demo cannot determine whether care is needed.

Report provenance

One-off AI-authored report prepared by ChatGPT from the bundled synthetic fixture. Cached offline for the pitch; no live API call, trained small model or diagnostic model is used.

No live API key is required to present this report. A future deployed interpreter or small language model is a separate implementation and evaluation step.